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# EDITED TRANSCRIPT

MRK.N - Merck & Co Inc at Bernstein Strategic Decisions Conference

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## OVERVIEW:

Company Summary

## CORPORATE PARTICIPANTS

**Robert Davis** *Merck & Co Inc - Chairman of the Board, President, Chief Executive Officer*

**Dean Li** *Merck & Co Inc - Executive Vice President, President - Merck Research Laboratories*

## CONFERENCE CALL PARTICIPANTS

**Courtney Breen** *Bernstein - Analyst*

## PRESENTATION

**Courtney Breen** - *Bernstein - Analyst*

All right. So we might kick off our conversation with Merck today. My name is Courtney Breen. I am the US biopharma analyst here at Bernstein. I am very privileged to have Rob Davis and Dean Li from Merck with me today. I'm extra privileged because I used to get to work at the company. And so this is hopefully going to be a wonderful conversation and hopefully enlightening for lots of investors. We're going to start and let Rob set the time set a little bit of this conversation out. And then we'll spend some time diving through questions that I prepared.

Just as a reminder, there is the Pigeonhole app, please do add questions into that so that we can integrate them into the conversation today and make sure that all your questions are answered and you all away feeling like you've got a really good understanding of the company.

So with that, Rob, I'll hand over to you. And take us away.

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**Robert Davis** - *Merck & Co Inc - Chairman of the Board, President, Chief Executive Officer*

Great. Well, thank you for having us, and thank you for everyone who's in the room and those who are listening online. Obviously, it's an interesting time right now in the pharmaceutical industry, and I'm sure we'll have ample opportunity to get into that. But what I'd like to focus on for just a minute is kind of the journey we've been on at Merck. Since I took over as CEO going on just almost four years ago, CEO -- did I say CFO. -- all right. I used to be CFO. Caroline thinks I still try to be CFO. I try to stay away from that.

And then obviously, Dean came into his role as Chief Science Officer and Caroline stepping in as CFO. And at that time, if I go back to 2021, what we set out to the company was a simple strategic framework tied to how are we going to deliver the purpose of the company anchored in four key priorities. And I won't get into all the priorities, but I'll get into the most important priority, which is the priority in the upper left still on the slide we show today, and it's guided us consistently with focus and intent ever since.

And that was we needed to invest in, accelerate and augment our pipeline because we have both the privilege of having what likely will be the largest pharmaceutical put aside maybe some of the COVID vaccines in history, which has had a profound impact on patients with cancer what's created an interesting challenge, which is how do you manage through that.

And so we knew we needed to move with focus and urgency to address that, and we've been dedicated to that ever since. And I can say that I'm quite proud of the progress we've made. If you look today, we have nearly tripled the number of Phase III assets we have in that time period. We now have over 20 assets, 20 unique assets, almost all of which have blockbuster potential in Phase III so in late development that we'll be launching over the next three to five years, two of which, frankly, are already in the midst of launching WINREVAIR and -- that are both off to a strong start. So I feel very good about the late-stage pipeline and where we are. And then we've seen good progress in our early-stage pipeline.

We have different programs that are going to be moving from Phase I into Phase II, and you'll have soon line of sight to those as we move forward over the next couple of years. So a lot has been done within the pipeline to invest in it. We put all of our resources behind this. We've looked at every opportunity to accelerate it. We can give examples where we've done that. Enlicitide, which is our oral PCSK9, we think will be a first-in-class, best-in-class treatment for arterial sclerosis or lipid lowering.

We've greatly accelerated that program by a few years from where it originally was where we were at a starting -- basically a standing start a few years ago. So more to do. I in no way believe we're done. But I look and feel that if we can do in the next three years, what we've done in the last three years, we have a real possibility and to achieve what I aspire to do, which is to grow through True to LOE.

Obviously, that is a tall hurdle, but we see it in our headlights to maybe be able to do that. As I said, more to do, we need more business development so the augmenting is going to continue as we've done. We've deployed over \$70 billion into business development over the last few years and brought in many meaningful assets. So more activity -- but as I sit here today, given the strength of the balance sheet, the strength of our margins, the legacy we have in oncology, the portfolio we can build from and now as we move into a much broader and diversified portfolio of products, we're well positioned.

Obviously, a lot in the macro environment we're going to talk about and have to deal with it. I believe as long as we stay focused on the innovation is the core of who we are that's the path to long-term sustainability, and I feel very good about what we're doing to this point.

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## QUESTIONS AND ANSWERS

**Courtney Breen** - *Bernstein - Analyst*

Fantastic. And I think that frames up a lot of the Merck story. And for a second, we're going to zoom back out and talk about some of these sectoral pressures because I think these are kind of the moment and certainly impacting where all stocks are trading for the entire sector right now. In many ways, this is a multiheaded beast. We've got tariffs. We've got MFN, we've got Medicaid cuts. We've got PBM reform. We've got HHS changing strategy and organizational structure. How do you rank some of these on a relative risk and size of impact basis. What is it that you focus on when it comes to the noise that we're all living through.

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**Robert Davis** - *Merck & Co Inc - Chairman of the Board, President, Chief Executive Officer*

Yeah. So I would say, really, from the -- what I see is the big policy questions, of the whole list you laid out. Clearly, first would be MFN, the reference pricing executive orders that came out for most favored nations what we can get into that because I think there's still a lot of questions around what that will be. But as a policy matter, that is obviously significant. And then the tariffs and how do we think about that and addressing that to make sure we can onshore and reshore manufacturing, the structural elements of that.

I would say on that one, we can go into it in more detail. We've made great progress. Actually, we, as a company, we're already moving down a path to move to a strategy where we would do US for US manufacturing, Europe for Europe, Asia for Asia, coming out of the pandemic because we recognize the need for more resilience in our pipeline and our supply chain of how we bring our products to the patients. And so that was the path we were on.

And in fact, coming out of 2018 with the Tax Cuts and Jobs Act, that allowed us to access foreign capital. We did deploy that largely into the United States. We've deployed \$12 billion between 2018 and 2024, all into manufacturing capital in the United States as well as building new R&D facilities and building pilot manufacturing, which is really what you need to do the conversion from clinical into commercial.

So a lot of investment in that \$12 billion. We've just announced a groundbreaking of a \$1 billion facility in Wilmington, Delaware to be an all-purpose biologics facility and we feel very good about that. And we've deployed other capital with the intention of having an incremental \$9 billion plus as we look out between now and 2028. So we're, I think, well positioned to be able to produce in the United States, more to do but I think we're as positioned there as we can be. And then as you look at kind of everything else, the rest, you talked about FDA changes, all of what's happened to CDC and NIH.

In the near term, those are obviously important I would say we are not seeing those have yet in the short term, the direct impact to us. I think there's some long-term structural questions that those things could have depending on how they play out to the industry and the broader ecosystem of

how R&D drug development is done. But in the near term, it's not having much of an impact. And probably the most important thing for us is what we're seeing with the FDA who is continuing to move on all of our important programs.

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**Courtney Breen** - Bernstein - Analyst

Fantastic. That's really helpful to understand. And I think you really hit on MFN and tariffs being critically important. So maybe just to spend a minute on those. I think Merck has actually been very transparent in some of the actions that you've taken to mitigate the near-term tariff risks. And I think we heard at the quarterly earnings, the work that you've done on KEYTRUDA in terms of bringing that inventory here into the US. Was that a hard decision to make? And can you talk a little bit about kind of -- is it just KEYTRUDA or are there other parts of the business that you've been able to kind of minimize that potential impact of tariff.

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**Robert Davis** - Merck & Co Inc - Chairman of the Board, President, Chief Executive Officer

No, I appreciate the question. No. So as -- and maybe this gets to a broader approach we're taking to a lot of what's happening right now in the broader environment. I tend to try to first remind myself and remind the team, well, this feels like a lot and a lot has come at us. And it feels like we've been living in this for a while. The reality of it is, it's 100 days. We've talked about the fact I mentioned earlier, I was in a meeting and someone said, can you tell me all the strategic changes you've made because of MFN and that was 1.5 weeks ago. So -- and we are in a business that operates on 10-year-plus time lines.

So what I've said to the team is what we can control is what's in front of us, which is executing on the pipeline we have, continuing to make sure we bring forward breakthrough products that if we can commercialize successfully and win in the marketplace, we'll make a difference for patients and will give us a sustainable profile for the company. So stay focused on that. Where we see no regret moves. Let's make them? I would put your question on KEYTRUDA as a no regret move. We already we had, as I mentioned, started to strategically shift to manufacturing.

We had initially not intended to bring all of the IV production of KEYTRUDA to the US because of the fact that it's approaching its LOE, and we felt like we'd be better to focus on the new products coming. As the situation unfolded, though, we saw the opportunity to pivot and to be able to know that if we make an adjustment and just look to bring in first contract manufacturing to fill some of the gap and then build the capability at the Delaware facility I mentioned, which is a broad-based biologics facility. It was easy to bring KEYTRUDA into that for both drug product and drug substance. So that solves the problem longer term. And then the shorter period of time, we brought inventory in of finished goods, which have us positioned to be able to source for 2025.

And importantly, we've also continued to both produce drug substance in the United States. We have two contract facilities where we do a small amount of production, most is done in Europe, but we do have some in the US. So we're going to leverage that drug substance from our US contract manufacturing for 2026. And then we've already signed up a drug product contract manufacturer. So we'll be producing both drug product and drug substance in the US beginning in 2026 and then set up for the long term. So that's an example of a no-regret move. There is some cost associated with it. We will mitigate that cost as much as we can.

But structurally, it fits with what we were trying to do anyway, and it satisfies the desire to see more production of key products in the US. So that's one where I felt like it was not a hard decision, and we've made the same decision for the rest of our pipeline. So where we can, we will source from the US. And we were already down the path today where the majority of our new products were already intended to be sourced in the US. And as it relates to IP, for most of our new stuff, frankly, the IP is largely in the US. So that's not an impediment going forward. For KEYTRUDA, the IP is not in the US. There will be a little bit of tax leakage, but again, that's something I think we can continue to mitigate and optimize around.

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**Courtney Breen** - Bernstein - Analyst

Understood. That's really useful to understand kind of the steps that you've taken and certainly, the various trade-offs and no regrets moves feel like the right thing to do in this uncertain environment. On MFN, because this is the other really big one, this is drug price reductions here in the US. Have you had any direct engagement with the administration since we got the executive order and then the CMS press release last week, there

seems to be light on details so far. And so all of us, that are out there looking and saying, well, do the companies know more than we do? Or do they only know as much as we do. Do you have any perspective you can share on --

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**Robert Davis** - Merck & Co Inc - Chairman of the Board, President, Chief Executive Officer

I'd say largely my estimation would be we know about the same as what you do. We have had -- maybe stepping back more broadly, we have had ongoing discussions with the administration, really going back to when they were transitioning into office including myself, with members of the cabinet, with the President himself and others with members of Congress, both the House and the Senate, particularly on the Republican side to understand the agenda and what they were trying to do so we've had a continuing dialogue as Merck and as the industry. And I would say it's been very constructive.

Clearly, this administration has strong priorities they want to achieve. But my own experience, and I think the experience that we've seen is if you're working to try to work with them on those priorities, they're willing to listen and they're willing to take your input. So that has happened. I don't want to get into the specifics of what's happened with the executive order itself and those discussions. But needless to say, as Merck we remain willing and interested in engaging ongoing with the administration on the MFN issue on trying to find a path to meet what we agreed to in principle and maybe that's important to say, both as Merck and as the industry.

In principle, we agree that the prices in the United States need to be addressed. We think the best way to do that is an industry and I'm sure it's a broken record to everyone else who's been up here, but it's true, is for every dollar spent on a pharmaceutical, over \$0.50 goes to someone in the middle. They have nothing to do with the innovation. They take no risk to the discovery of the drug. They take no risk to the development, no risk to the manufacturing. They negotiate price and they distribute. Find anywhere else in the world where the person who is a negotiator and distributor takes \$0.51 for every dollar.

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**Courtney Breen** - Bernstein - Analyst

We take about 2% or 3%.

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**Robert Davis** - Merck & Co Inc - Chairman of the Board, President, Chief Executive Officer

Yeah. I mean we need to address that. That's the fastest way to bring down prices in the United States. And by the way, that changes the economics of our industry because we only get the \$0.49 today. So my view and the view in the industry is if we do that, we both can preserve the innovation engine we have, which is a world leader. We lead in this, we're an exporter of this and also address the price issue in the United States.

So that's half of it. We also, in principle, agree that we have to do something about the fact that there's an imbalance between what the US pays for drugs and what the rest of the world pays that's acknowledged. We think the best way to do that is both through our voice as an industry, which we're doing, do everything we can to communicate the need to see this addressed.

But frankly, it's going to need beyond just the industry. You need government to government action because in most of the countries outside the United States, their social systems where you're negotiating with the government, they set the price using health technology assessments. It's not necessarily a negotiated price. It's largely a set price and we need help, and we think through trade and tariffs, there are ways for the government to help us. I think you've seen that being reflected and even in the executive order itself there was a reference to having the Secretary of Commerce do just that. So we support all of those actions. So across that, we're willing to engage and try to solve those problems.

Let's find the best path to do it that preserves the industry but addresses what we need, which is lowering the prescription costs for patients at the pharmacy counter and getting better balance for the value of the innovation we bring globally.

**Courtney Breen** - Bernstein - Analyst

And it certainly sounds like the industry is working in a concerted way together to do this. I think we're hearing very similar messages across all of the session. Dean, I think we heard just from Rob that -- and this is the final broad question before we really get into the Merck story, but we heard from Rob that perhaps there hasn't been any disruption to your interactions with the FDA or other pieces of the agencies so far. But is there anything that you're looking out for that would start kind of waving red flags and say, hang on, this is -- this is getting a little bit close to comfort. We're going to need to start making sure and reinforcing those engagements.

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**Dean Li** - Merck & Co Inc - Executive Vice President, President - Merck Research Laboratories

There's a lot of statements that are made and it sometimes can distract people from actually what's happening on the ground. And so far, when we look at it, I mean we have important -- as he's talking about, these launches are important for patients, and it's also important for Merck. But we have a pembrolizumab, which we think is really important for patients. We have not seen any issues with the PDUFA date that's been laid out. We have clesrovimab, which is an antibody against RSV, which we think is really important for the US health system. We are in engagement and relationship with the FDA. We have not seen any movement in that PDUFA date. So those are all great examples.

We've talked about later doravirine, which we think is critically important in laying out a new anchor medicine for HIV that file is moving appropriately to the FDA. We'll probably be talking about whenever state we had really excellent data. And that data, we have indicated to people that we're very interested in getting the first time a label in PAH, where we're talking about mortality survival in this. We have conversations. All of that has not gone off track.

I think looking forward, what we'll have to just see is there has been the exit of some relatively senior people, that drives some degree of stability as to what the expectation is and we'll just have to see how much of that expectation because this is -- we set up trials now in collaboration with the FDA. And then all of a sudden, when you read it out later on, if everything dramatically change, you're like, you know -- and we have not seen that, but that would be something that we would keep our eyes out on.

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**Courtney Breen** - Bernstein - Analyst

Absolutely. I think that makes a lot of sense given the long cycle plan for not only the clinical trials but from discovery all the way through. You touched on a couple of drugs that I think we're going to dive into. But the first, let's start with KEYTRUDA because I think this is the big one. In an earlier presentation that I had today, I showed a graph that represented the fact that consensus peak expectations for KEYTRUDA back in 2016 were only \$5 billion.

Have then shifted over the next 10 years up to being \$35 billion in terms of the expectations of where that will be by 2028. There is obviously a large patent cliff that comes with a large blockbuster and you've been doing a lot of work to set up for that with subcutaneous and some of these other opportunities. Then can you remind us on the subcutaneous product kind of why this is a clinically relevant kind of opportunity?

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**Dean Li** - Merck & Co Inc - Executive Vice President, President - Merck Research Laboratories

We think this is a really important opportunity. One of the things that I think happened in the transition from '20 to '21 is Merck really pivoted to get into the curative setting in the early stage. And we have hopefully 10 approvals shortly. Four of them have OS. This is a place where our growth is but it's not just economic growth. This is where you could hire patients. So we think it's critically important. We think, especially in that patient population, but really in all patient populations, if you can give it subcu and if you could give it subcu in 2 minutes, that would be really important. No one would like to stand in queue at an infusion center. It is not the most uplifting thing for anyone who's ever had cancer.

And so we think this is really important, and it dramatically changes access in the inner cities and in the rural are. So we think this is really important, and that is moving forward. So it's an injection I can do Q three weeks or Q six weeks and it's 2 minutes. There is nothing out there of any other PD-1 that is like that where you're giving it in 2 to 3 minutes. We think this is important.

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**Robert Davis** - Merck & Co Inc - Chairman of the Board, President, Chief Executive Officer

Which is much smaller volumes.

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**Dean Li** - Merck & Co Inc - Executive Vice President, President - Merck Research Laboratories

That's why it can be given so much I mean I hate to say this, but the minute you talk about putting things through a port in all of this, and I'm an attending physician, the last person you ever want to touch a port, chemoport is an attending physician. That's the last person you want touching it. I can give subcu pembro.

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**Courtney Breen** - Bernstein - Analyst

The reduction in the risk. And Rob, I think on the subcu piece, you've also alluded to kind of concepts around how you're going to price this relative to the IV, can you talk us through kind of what that might look like at launch relative to where the subcu might be as you go through IRA and then as you go through the patent --

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**Robert Davis** - Merck & Co Inc - Chairman of the Board, President, Chief Executive Officer

So our strategy -- and it gets a little bit what Dean was laying out that given the unique portfolio that KEYTRUDA has and the number of indications, we now have 41 indications across tumor types plus two that are tumor agnostic and more coming. And as Dean said, in early-stage cancer, we have nine currently soon to be 10, 4 with overall survival the ability to try to bring value to patients and, in turn, continue to lead in cancer for us is unique relative, I think, to any drug that's ever come before because of the size of what it is and the strength of the data and the commitment behind the use of KEYTRUDA.

And so we very much are focused on getting the maximum number of patients who are eligible to adopt the subcu form at launch. We believe, if you look at the total patient population that is out there, 30% to 40% will choose to be on subcutaneous, either as the patient both and then working with their provider. It brings provider benefits where you're in clinics that are capacity constrained. It brings quality of life benefits to the patient, and we think that will be so important. And so we're quite confident in that ability to drive the option but we recognize the one thing that could stand in the way of that is if access is limited because of how we price.

And so we're going to price for per share, we're going to price to drive the volume, which means we are going to price when get into specifics, but let's say the price we would expect to generally follow the IV price because that's what's going to ensure the maximum adoption over time. And then that would be true as we move into the IRA and potentially as we move into the launch of the LOEs as well, always focusing on, first and foremost, maximizing access and adoption and value under the curve because we think that can sustain a tail for the benefit of Merck while bringing in a significant medical benefit for the patients that are counting on it.

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**Courtney Breen** - Bernstein - Analyst

Absolutely. And I think that concept that you just laid out is really important because if your price on your subcu ends up a lot higher in that post patent period, your volume just switches back to the IV setting, which becomes then a highly competitive space and much more challenging to defend. So thank you for walking us through that. I think WINREVAIR is a particularly exciting opportunity that kind of mid launch at the moment, we're seeing kind of real patient impact with this product as well.

What drives the growth ahead as you think about kind of moving -- continuing to move. And I think if I remember in our models, we see kind of winter being about 10% of your overall revenues by the end of our forecasting period. How do you think about kind of that future and trajectory with winrevair. And what dimensions do you push on to achieve it?



**Robert Davis** - Merck & Co Inc - Chairman of the Board, President, Chief Executive Officer

Yeah. Maybe I'll start and then Dean can add from a clinical perspective. So right now, the launch is going very well. We had high expectations coming into this, we're meeting those expectations. So I feel like we're -- we were aggressive and we're delivering against that. If you look at where we are today, we're still fairly early. So we're still primarily focusing on bringing the sickest patients and those would be triple therapy patients, patients who were on therapy, looking at really the sickest you can see out there. But the ability now increasingly to see other doctors start to move into with people on two drug regimens, so they've come off the prostacyclin and that is starting to move.

So the potential of the ramp is still strong for trended growth. We're not expecting an inflection point, hockey stick. It's we think steady growth. And what drives this is the experience that the physicians are having. They're increasing confidence that the safety profile is proving to be quite good. We continue to have very good results from a safety perspective. We've started to publish increasingly the real-world evidence around what we're seeing on the safety profile, including we're going to -- you're going to see extended data from -- ZENITH study, STELLAR study and how we track these patients longer term. That is all looking good.

And then obviously, as we start to see the readout from, for instance, ZENITH, which was unbelievable, the results that came with ZENITH. And maybe -- because I know there are some generalists, pulmonary arterial hypertension is not well known. It is a devastating disease. It tends to hit women more than men and it hits them in midlife, so you're often dealing with child-bearing aged age-bearing women. And on average, if you're diagnosed with this, one, it takes a long time to be determined this is what it is.

But effectively, you're suffocating because you cannot get blood pumping enough because you're getting so much attention into hypertension, you will into the lungs and into the heart. 50% roughly will be dead within five years. mortality is high. And this drug is the first drug through the ZENITH study to show a mortality benefit, all cause mortality benefit, statistically significant benefit through the ZENITH study that was stopped early because of overwhelming efficacy.

And that's powerful because obviously, if you're a patient, even if you're a patient who is on therapy and is feeling okay and is being steady at the point you're at, you have to challenge when do you need to be put on to WINREVAIR because if it brings the mortality benefit, you would assume you'd want to be on sooner than later. And if you look at the curves in the study, they separate almost immediately. So it's very important to understand that and we expect, hopefully, we'll see.

We had to stop the HYPERION study as well because of the same issue. And it would be unethical to not give patients WINREVAIR because of the results we were seeing. And that's important because that was an earlier population of patients earlier in their disease progression. And so it helps reassure physicians that this is for the less sick patients as well as the sicker patients. So all of this is building the momentum that drives us to believe this will have steady growth as we move forward. And that's before we talk about things moving maybe broadly into a PH-related heart failure that Dean can speak of. But Dean, I don't know if you maybe if you want to go to --

**Dean Li** - Merck & Co Inc - Executive Vice President, President - Merck Research Laboratories

I would just emphasize people are going to later on ask about BD. We like BD that gives us the access of doing something first, best and next, whether it be IBI or whether it be Prometheus or whether it be Acceleron. And with Acceleron with WINREVAIR, you see it. This is the first time any medicine that isn't related to vasodilation, which is all the other medicines, but it's focused on the basic genetics of the disease. This is the first one. It's the first active in signaling inhibitor.

And I would remind everyone, I don't know of any other active and signaling inhibitor that is in any clinical trial anywhere in the world. So this is first. It has given best results. This is the first time a trial has been stopped early because of overwhelming efficacy. This is the first time that people have had hard outcomes and this is the first time that I know that not only did the trial get started for Zenith, but the steering committee refused to proceed with the other trial. I've never been in that situation, not even with KEYTRUDA. So this is first best next, it reformulate the field.



One of the things that are next is that we're going to experiment and see whether there are certain patients with pulmonary hypertension who don't have pulmonary arterial hypertension but have pulmonary hypertension due to heart failure. And we'll see those results sometime this '25, '26 to guide whether we go to Phase III. And so we're excited and eager to see those results to figure out whether we can even do more good for even more patients.

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**Courtney Breen** - Bernstein - Analyst

And those results will be quite instructive because the biology is a bit different, right?

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**Dean Li** - Merck & Co Inc - Executive Vice President, President - Merck Research Laboratories

It is -- the biology is different and the patient population with heart failure that we've taken is the one that, from a physiology standpoint looks similar to PAH. It's not the broader heart failure, but it's a relatively smaller patient population among the heart failure, but it is a population in heart failure who have the mortality and the life expectancy and morbidity similar to those patients with PAH. The unmet need is tremendous.

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**Courtney Breen** - Bernstein - Analyst

Perhaps just jumping around because we're on cardiovascular. And I know in an earlier meeting that I got to experience and you mentioned it before -- which is your RPC is an exciting readout that's upcoming this year. And I think it's one of the key markets, key milestones that many investors are looking at and waiting to see. Can you talk a little bit about what this ramp and opportunity might look like?

Let's assume that this is positive. We look at the other PCSK9. And what we've seen with them has been a very slow build and that penetration is very, very low in the market. Cardiovascular generally takes a long time to launch a new product. Will that be true for this particular product? And kind of what is it about this product that might mean that we end.

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**Dean Li** - Merck & Co Inc - Executive Vice President, President - Merck Research Laboratories

Let talk about the commercial ramp up, but I'm very excited about this. I am a cardiologist. I did internship residency when statins came out, and I wasn't attending when the first PCSK9 antibodies came out. And those first antibodies came out at 15,000 a year. which just so that you know as a cardiologist, we used to snicker and say, you're charging the same as what we charge for percutaneous coronary intervention, are you what people want is they want easy access to the lowering of the LDL cholesterol.

We wanted democratize the PCSK9 pathway because that is the most potent way to lower LDL cholesterol. So we have a medicine that we dream that this medicine can be given in the morning, no different than your statin, your aspirin, you're antihypertensive in the morning. You don't go to the hospital to get a shot, you don't do anything like that. And it will lower your LDL 60%, just like the antibodies because what this medicine is, is we have the technology to take an antibody and reshape it using a different technology to stick with a peptide and make a biologic in a pill. So we're excited about this for the broader implications.

And the concept that we want is not just to lower by LDL by 60%, that's the biomarker data that will come out this year. but our aspiration is not just to be first and best in terms of that. But we intend to have an outcomes trial that will read out two to three years later that has an outcome of a reduction, not a 15% cardiovascular outcomes, which is what all the antibodies for PCSK9 have achieved we intend to achieve and so that's very exciting because 70% of individuals, especially in the United States, where cardiovascular is still the number one do not have their LDL in the right place. But commercially --

**Robert Davis** - Merck & Co Inc - Chairman of the Board, President, Chief Executive Officer

Yeah. I mean the commercial opportunity, obviously, we think is significant I think some important points being hit on 70% of patients despite being on a statin don't reach goal. Cardiovascular disease continues to be the number one killer, 85% of which is related to people having problems with atherosclerosis, or hardening of the arteries and blockages. So there still is a huge unmet need. And if we can bring a drug that is oral, that you can take and importantly, price it competitively, because we -- I would say, something we brought and where we focused our capabilities is in two places. Obviously, this is using macrocyclic peptide technology for the discovery which, as Dean said, is new, and it's important. And this is one of the most complex areas that has been used with the oral PCSK9.

But frankly, others have that capability. But what we think is unique is we have invested equally in our manufacturing CMC capabilities so that we can produce macrocyclic at low cost. And the biggest -- one of the biggest barriers for macrocyclic have been they're very expensive to make because they're highly complex to produce. We've been able to drastically reduce the number of steps it takes in the manufacturing process through know-how that is big to us. and that will allow us to be very competitive in how we price, which we think is -- that's the democratization that Dean is speaking to, both in the United States and around the world because if you're going -- if we come out in a lower price and drive for volume, to address the need of the world, we think that's been one of the biggest barriers to why you're not seeing adoption that we can overcome given the clinical benefit that he's talked about.

It's important to note that when we launch, we'll be launching with the biomarker data, which we'll see the readouts of those Phase III studies, three of them this year, later this year. But as we look at that because of the fact that this is a biologic in a pill that follows the key structural elements of the monoclonals that are out there, we think that our ability to get a favorable label consistent with the biologics will be there, no different than what happened with inclisiran.

And then obviously, we will follow on with the outcome study in the next couple of years as being said. So we expect this to have a strong launch from the beginning and then to carry it through with the outcomes data, we'll be probably two years ahead of any competitor. And our expectation is we'll be having outcomes data as you start to see competitors come in. But then I think what Dean should comment on is what's next because we're not stopping with the mono just it is a single agent and once you speak about the combination.

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**Dean Li** - Merck & Co Inc - Executive Vice President, President - Merck Research Laboratories

So there will be some obvious combinations that you would combine that you would think about a statin, you would think of other non-statin LDL cholesterol medicines. But probably really exciting to us is they are the cohort of patients whose LDL is high and have a high LP(a). The way that I treated patients right now is I lower their LDL to 0. And there could be evidence in the next year that lowering -- will also be important.

So naturally, you sit there and you go the perfect medicine for this patient population could potentially be a PCSK9 LPA combination. And so as I talked about having PCSK9 with towards 20% cardiovascular outcomes reduction in the high Lp(a), we could dream of having a pill for that patient population, we're driving the cardiovascular risk down by 30%. I mean those numbers are the numbers I like here, 20%, 30% reduction in cardiovascular outcome.

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**Courtney Breen** - Bernstein - Analyst

And you can certainly hear your passion of cardiologists coming through as you talk about this topic, perhaps jumping to combinations and kind of the rest of the plan. TL1A, I think, is an interesting one. This is your an immunology product that you have in the pipeline that is in Phase III right now for your -- the IBD space. I think in many ways, you hear from some of the other companies kind of we're only going to bring forward TL1A in a combination. How do you think about kind of the path forward? And perhaps is this a place for M&A for Merck going forward to find partner molecules to the --

**Dean Li** - Merck & Co Inc - Executive Vice President, President - Merck Research Laboratories

So just so that we step back, the type of market or biology that TL1A would play in is the old place, for example, HUMIRA and SKYRIZI and STELARA. So there's TNF as an important node, IL-23 as an important node. What we're hoping to be is, again, I hate to be a bad recording, first best next. We intend to be the first TL1A and we believe we will be the best TL1A, and that TL1A node will be as important, if not more important than IL-23 and TNS so we are advancing with speed in inflammatory bowel disease, and we will be opening it up in other diseases over this year.

The miniature can be first and best there, you immediately go next. And then next is you would begin to combine with the other major nodes such as IL-23 or TNF or others. You can achieve that by doing combinations that you're talking about, but you could potentially also achieve that by making, for example, bispecific antibodies.

And you would also immediately start asking yourself if you can make elicitor and a PCSK9 antibody in a pill, could you do the same thing here as well. So it's creation where we do BD where we can have the hope of being first best and it opens up a pipeline of next.

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**Robert Davis** - Merck & Co Inc - Chairman of the Board, President, Chief Executive Officer

And we have internal agents already to do for the combination as well as continuing to look external.

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**Courtney Breen** - Bernstein - Analyst

On M&A specifically and more generally, I know you guys have done a lot of deals recently in China, and it seems like you've probably got a pretty good team on the ground that are kind of making sure you've got deep view in there. Can you talk a little bit about what are the parameters that kind of feel like a good deal for them to consider at this point in time compared to the last few years.

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**Robert Davis** - Merck & Co Inc - Chairman of the Board, President, Chief Executive Officer

Yeah. Well, the overall, the parameters we've brought really haven't changed fundamentally. We continue to be driven first and foremost by the science. And always asking the question, is there a scientific need that solves an unmet need for a patient that fits within our portfolio and then does it align with value or not. And if we find that word, there's new science that addresses unmet need strategically, it fits and we add value, that tends to be where we move. And we've been very strategic in thinking about our portfolio to diversify. If you go back a few years ago, we were the KEYTRUDA company.

Today, we are a broad-based oncology company. And increasingly, we're going to be a cardiometabolic company, an immunology company, an ophthalmology company, a vaccines company. So we've been very thoughtful and leveraging our position to grow out, and we've done that both internally and through business development. The sweet spot of where we've been looking has been that \$1 billion to \$15 billion range with the willingness to go higher but always if it meets the scientific parameter I just laid out. We're not going to do a deal just to do the deal. It's got to be -- bring us great science that brings sustainable value for us long term.

We've moved into Phase I, Phase II. We've done stuff in Phase III, and we're open to doing things that are commercialized. So all of that is areas we can be interested in. If you look at what we've done recently in China, those deals were because we saw opportunities to leverage capabilities in the first best next that Dean just walked through. And in many of these cases, they're giving us the opportunity to go to the next, what's the combination drug. And what we find is high-quality science, good assets, and we go for those wherever they are, if the science is there, if the clinicals have high quality, and we feel confident in that after diligence, we're comfortable with the intellectual property.

And in the case of China that we can bring the assets out of China and then do the studies, the ongoing further work, commercialization for the rest of the world outside of China. So that's important as we think about the Chinese assets but I've been asking other settings, are you more leaning towards China than everywhere else? The answer is no. It just happens to be that the recency bias, the last deals have been there. It's just because that's happened to be how it played out, not because we've changed our focus, and we're looking globally for further opportunity.

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**Courtney Breen** - Bernstein - Analyst

We have got very far into this conversation without talking about ADCs and kind of your pipeline. I know we're going to have a lot of talks and a lot of airtime on oncology over the weekend and over the next few days at ASCO and I look forward to seeing the team there, although there was an announcement this morning, patritumab deruxtecan, our ADC, can you just as a headline, let us know, is there a read across to the rest of the ADCs in terms of either safety or efficacy that we should be aware of? And we can definitely focus on the rest of oncology while we're at ASCO over the weekend business.

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**Dean Li** - Merck & Co Inc - Executive Vice President, President - Merck Research Laboratories

Yeah. So I would just emphasize that for us, and you'll see at ASCO, I mean antibody drug conjugates in the clinic, five of them are pretty deep in the clinic. The first one that everyone focuses on is -- which is TROP2 ADC. It's in four clinical trials. In Phase III -- I'm sorry, 14 Phase III clinical trials 9 of them, if you look at them, it's in a tumor indication that we will be first. And the other five, it's a place that we think we can be differentiated. So that's with Kelun and they have been a great partner. With Daiichi Sankyo, there's three of them. The most important indication for each three of them is the HER3 ADC that you spoke about.

And our key interest is in breast cancer that's where we focus that HER3 ADC. So there was an announcement in relationship to lung, but that her ADC, that's where we're focused on. B7-H3, we think that we can change -- we think we can bring to small cell lung cancer, what we did to non-small cell lung cancer with KEYNOTE-189. And we -- is advancing a B783 small cell lung cancer. But instead of using a PD-1, we're using a T cell engager, which is BML 3 T cell engager, and that's advancing very fast. And with the Daiichi Sankyo, we also have moving forward CDH6 in ovarian. There's many others.

And then I think one of the ones that I think some people have lost track of is the ROR1 ADC, which we're now moving quickly through so those are five, we think that the antibody drug conjugates are really important. We're the company that combined pembro with chemo. We are the company who should combine a PD-1 with next-gen chemo, which is antibody contend we intend to continue to do that.

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**Courtney Breen** - Bernstein - Analyst

Fantastic. Super helpful to understand that context. I think we'll get into a lot more detail over the next few days. I want to make sure that we had a couple of questions in here and one connects to another question that I wanted to ask, which was about margins and kind of where margin goes for Merck because I think there's a lot of leverage that comes from KEYTRUDA and you've got some other drugs that are growing like winrevair that have royalties associated with them, too.

And then the question that came through on the Message Board was referencing the contract manufacturing that you just spoke to Rob earlier for KEYTRUDA, should we expect the hit to margin for KEYTRUDA with that contract manufacturing and those new contracts. So just can you talk a little bit about the pushes and pulls on margin how that might evolve over the next 10 or --

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**Robert Davis** - Merck & Co Inc - Chairman of the Board, President, Chief Executive Officer

Yeah. So obviously, we've been very focused on trying to drive the strongest margin possible over the last several years. And frankly, part of why we did that was to make sure that we always are in a position, depending on what would happen in the US pricing environment to be able to absorb pricing hits and be able to always fund innovation because we continue to believe the best path forward for our company long term is to focus on the science and focus on innovation who we are and we can -- putting the macro environment aside, people will always want a cure for the next disease that they face it.

And we want to be there to deliver that and there'll be value for that. So that's very important. What's driving our margin is, as you said, obviously, product mix. We've also done very, I think, good cost management. We have the highest percentage of -- if you look at from an R&D I should maybe

flip it there, I'm going to go to the direction. But SG&A, as a percent of sales, we're the lowest or probably near the lowest in the industry. So we're the most efficient in SG&A. We're also the most efficient in cost of goods sold. The contract manufacturing is not going to have a major impact on that. It's in the mix of what we do. So that should not worry anyone. As we look forward, with the mix change as KEYTRUDA comes off patent, clearly, that will create a margin headwind.

And but how much of a headwind will depend on the rest of our product mix. And I would say some of the products we're bringing into new out do have royalties, so they have -- their margins are a little bit lower, but some have very high margins because of the way that those contracts were signed up for just -- for example, if you look at the Daiichi Sankyo agreement, the profit sharing that happens there all flows through basically revenue as net revenue. So the gross margin of all of what we do for the Daiichi products, same thing for Moderna 100% because now they're below the gross margin line, you have the R&D expenses and everything else.

So obviously, those are very good margin products. Now that's because the costs are buried into the net revenue. It's the way you book alliance revenue and a partnership. And then we have others that would be wholly owned assets without royalties and then some with. So there's a pretty good mix and how that all plays out, I think we're going to have to see over time. And then clearly, in the short term, we're going to invest behind the R&D we need to be able to deliver the growth of the pipeline we see.

A lot of the margin benefit we've seen flowing through gross margin, we've been just redirecting into R&D spend. And that gives us, obviously, flexibility longer term to think about that as well. So I think the margin will face the headwind, but how it plays out, we'll have to see based on the way the portfolio grows. I'm most focused on delivering top line growth, which I think if I do that with the product portfolio we have, I'm confident you're going to see earnings growth as well.

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**Courtney Breen** - *Bernstein - Analyst*

Fantastic. And maybe just to end, I know we've got about a minute left. The \$50 billion of that top line contribution that you've spoken to in the past from the pipeline that you have. There was a question that came through with kind of what are the error bugs that sit around that \$50 billion? How much rain --

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**Robert Davis** - *Merck & Co Inc - Chairman of the Board, President, Chief Executive Officer*

Okay. Well, so that \$50 billion plus just to give context to what it represents, and this is obviously before we take into account what happens with post favor Nation and everything else so that people understand that -- but the \$50 billion plus is the revenue potential nonrisk adjusted that we see coming from oncology, which we think has greater than \$25 billion in potential coming from the vast suite of assets that Dean walked through plus others. It's coming from our cardio medical business anchored with the oral PCSK9 as well as winrevair and then -- which is our GLP-1 glucagon coagonist that's greater. That's approximately \$15 billion.

We have greater than \$5 billion from immunology and the TA platform that Dean talked about earlier, greater than \$5 in HIV coming from multiple assets we have there that we didn't even get to touch upon that we're excited about. We have one in ophthalmology, which we think is a multibillion-dollar opportunity based on the wind agonist, a first in class, best-in-class deal, very similar, Dean calls it the winrevair if you will, of ophthalmology, which we hope to deliver on. So all of that -- and then beyond that, we have the benefits of what's coming from vaccines from business development. And then our early-phase pipeline, we have over 50 programs moving into Phase II.

Some of those because they're in spaces like oncology, could be delivering at the same time window mid-20s. Those haven't been included in animal health, that's going to more than double over that period. I need to look to my team because we have so much hard to keep it all on your head. The error bar around that, that you should assume is we're saying that's the -- if everything hits, but I think there's a misunderstanding that people view it as it's toggled on or off, take oncology, just use that, the \$25 billion.

That's across the 9 ADCs where we have 50, Phase III studies underway with -- between the ADCs, the tissue targeting agents, we have the precision molecules we have, the INT therapy in combination with Moderna across -- so if you look across all of those, the T cell engagers bispecifics it is a broad suite of assets that each have multiple indications.

So the way you can achieve the \$25 billion is through a whole host of different measures. So our error bar there, we feel is pretty tight because it's spread across so many assets. It's not one asset. It's many assets. I think almost that's the trouble of our story, where we're misunderstood. We have so much. It's hard to get your arms around it. But what you should hear from us and for me, our confidence in that pipeline is incredibly high.

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**Courtney Breen** - Bernstein - Analyst

Thank you so much. I think that's a great place to end. I really appreciate. Thank you Dean.

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